

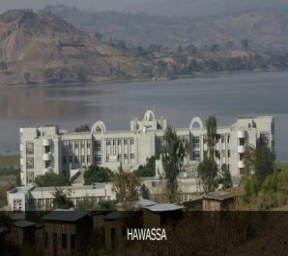


Hawassa University Comprehensive Specialized Hospital

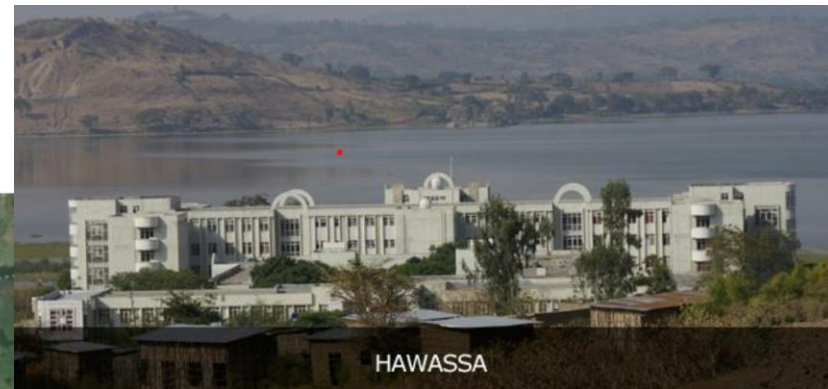


A CASE OF PROXIMAL GASTRIC TUMOR PERFORATION PRESENTING WITH GENERALIZED PERITONITIS WHILE ON NEOADJUVANT CHEMOTHERAPY

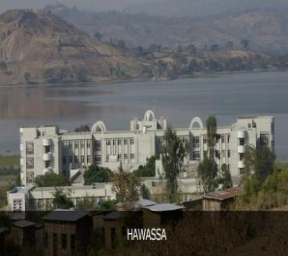
Dr. Siyasebew M
Cardio thoracic surgeon
Hawassa University, Ethiopia



HAWASSA UNIVERSITY, ETHIOPIA







History

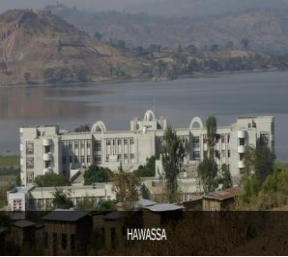


- 28 years old patient with known proximal gastric cancer presented exacerbation of abdominal pain of 36 hours duration
- He claim to have diffuse abdominal pain, three episodes of vomiting and fever.
- He was diagnosed to have proximal gastric tumor with confirmed adenocarcinoma 2 months prior to his current complaint

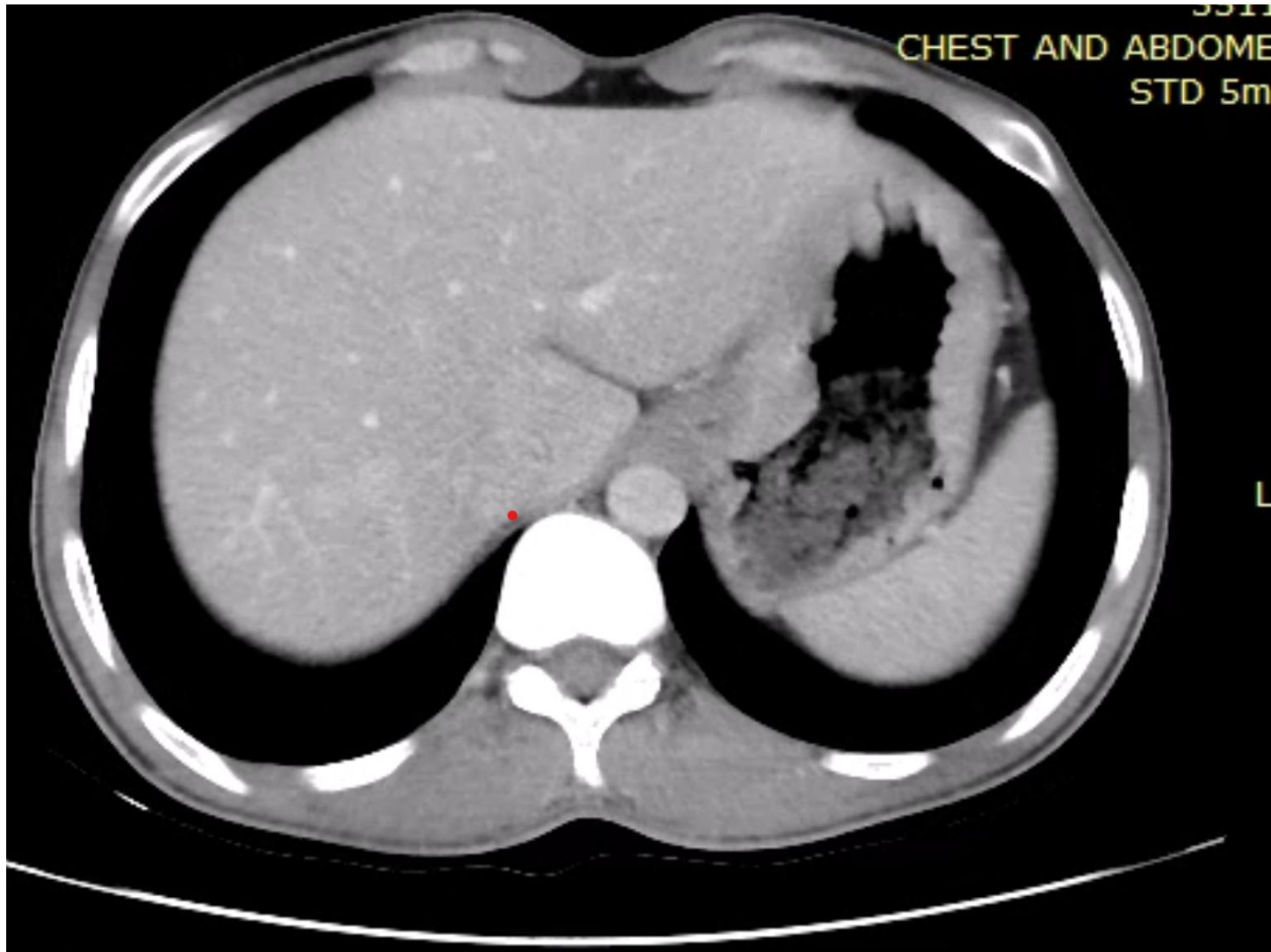
Chemotherapy

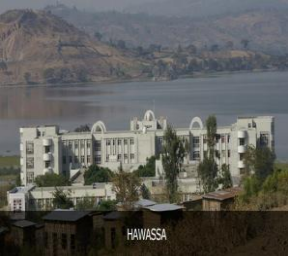


- He took two cycles of,
- 5FU 707 mg iv bolus, then 2123mg iv infusion over 46 hours
- Oxaliplatin 150mg iv over 3 hours
- Docetaxel 88mg iv over 2 hours
- Leucovorin 200mg iv



Preoperative Chest/abdominal CT scan

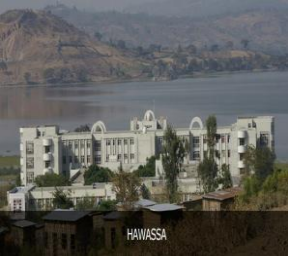




Workups



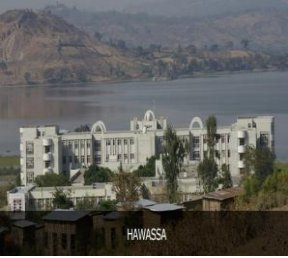
- Chest x-ray** - unremarkable
- Upper GI endoscopy**-
 - Normal esophageal mucosa
 - ulcerated mass noted just distal to z line, normal body and antrum,
 - normal D1,D2 and no ulcer
 - **Biopsy**-infiltrates of well differentiated adenocarcinoma



Clinical finding



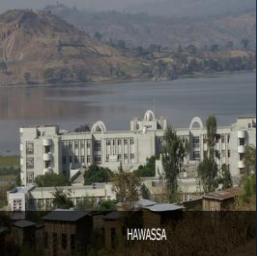
- He was evaluated in the ER and diagnosed with generalized peritonitis 2ry to viscus perforation
- Intraoperative finding:
- 1500ml of gastric content in the general peritoneal cavity
- There was proximal gastric mass involving cardia and extension along lesser curvature, GEJ was free



Intra-operative finding



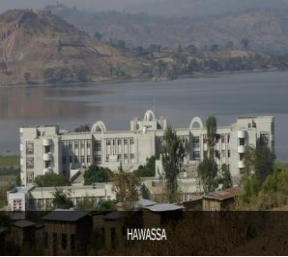
- There was 2.5cm tumor perforation along the lesser curvature
- No evidence of local adhesion, liver and rest of peritoneal cavity if free of metastasis.
- The gastric content sucked out, the defect was approximated and patched with omentum,
- He had smooth post operative course in the hospital and discharged on 7th day.



Post operative course



- Smooth 3rd post operative month
- Claim to gain weight, feeding well and wound site healed
- Did not resume chemotherapy
- **CBC ,LFT , RFT** - normal range
- **Serum electrolyte**-showed moderate hypokalemia which was corrected



Recent CT



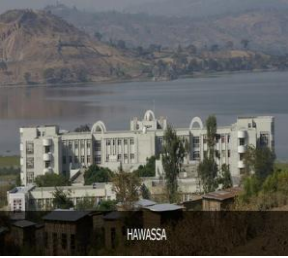
Final decision



- Elective total gastrectomy decided
- Initial exploratory laparoscopy was done on 28/10/24 for suspicious omental lesions on the CT scan
- Followed by total gastrectomy and Roux-en-Y esophagojejunal anastomosis in the abdomen
- Discharged home on 6th post operative day
- Pending final pathology



Key points



- Rare presentation but fatal?
- ?Direct toxicity(5FU, docetaxel)
- One stage versus two stage gastrectomy(short/long term outcome).